Gemeinsamer Bundesausschuss

Resolution

of the Federal Joint Committee on an Amendment of the Pharmaceuticals Directive:
Annex XII – Benefit Assessment of Medicinal Products with New Active Ingredients according to Section 35a SGB V
Dalbavancin (new therapeutic indication: acute bacterial skin and skin structure infections (ABSSSI), from birth to < 3 months)

of 20 November 2025

At their session on 20 November 2025, the Federal Joint Committee (G-BA) resolved to amend the Pharmaceuticals Directive (AM-RL) in the version dated 18 December 2008 / 22 January 2009 (Federal Gazette, BAnz. No. 49a of 31 March 2009), as last amended by the publication of the resolution of D Month YYYY (Federal Gazette, BAnz AT DD.MM.YYYY BX), as follows:

I. In Annex XII, the following information shall be added after No. 5 to the information on the benefit assessment of Dalbavancin in accordance with the resolution of 1 February 2024:

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Dalbavancin

Resolution of: 20 November 2025
Entry into force on: 20 November 2025
Federal Gazette, BAnz AT DD. MM YYYY Bx

New therapeutic indication (according to the marketing authorisation of 2 May 2025):

Xydalba is indicated for the treatment of acute bacterial skin and skin structure infections (ABSSSI) in adults and paediatric patients from birth.

Therapeutic indication of the resolution (resolution of 20 November 2025):

Xydalba is indicated for the treatment of acute bacterial skin and skin structure infections (ABSSSI) in paediatric patients from birth up to the age of 3 months.

1. Extent of the additional benefit and significance of the evidence

For the medicinal product Xydalba with the active ingredient dalbavancin, an exemption from the obligation to submit the evidence according to Section 35a, paragraph 1, sentence 3, numbers 2 and 3 SGB V was granted by resolution of 20 April 2023, as it is a reserve antibiotic within the meaning of Section 35a, paragraph 1c, sentence 1 SGB V. The additional benefit is deemed to be proven if the Federal Joint Committee have decided on an exemption for a reserve antibiotic in accordance with Section 35a, paragraph 1c, sentence 1 SGB V; the extent of the additional benefit and its therapeutic significance are not to be assessed by the Federal Joint Committee. In the resolution according to Section 35a, paragraph 3, sentence 1 SGB V, the Federal Joint Committee shall specify requirements for a quality-assured application of the reserve antibiotic, taking into account the effects on the resistance situation.

Children from birth to < 3 months with acute bacterial skin and skin structure infections (ABSSSI)

Additional benefit of dalbavancin:

The additional benefit is considered proven.

2. Number of patients or demarcation of patient groups eligible for treatment

Children from birth to < 3 months with acute bacterial skin and skin structure infections (ABSSSI)

Approx. 16 to 38 patients

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3. Requirements for a quality-assured application

The requirements in the product information are to be taken into account. The European Medicines Agency (EMA) provides the contents of the product information (summary of product characteristics, SmPC) for Xydalba (active ingredient: dalbavancin) at the following publicly accessible link (last access: 06.10.2025):

https://www.ema.europa.eu/en/documents/product-information/xydalba-epar-product-information_en.pdf

The requirements for a quality-assured application of dalbavancin apply to the new therapeutic indication approved as of May 2025.

Dalbavancin may only be used in paediatric patients from birth up to the age of 3 months for the treatment of acute bacterial skin and skin structure infections (ABSSSI) if there is evidence or, in exceptional cases, strong suspicion that the infection is caused by methicillin-resistant Staphylococcus aureus (MRSA).

Dalbavancin should be used with caution in patients with known hypersensitivity to other glycopeptides due to the potential occurrence of a cross-reaction. If an allergic reaction to dalbavancin occurs, use should be discontinued and appropriate therapy initiated.

For restarting treatment with Dalbavancin, consult a specialist with an additional qualification in infectiology, a specialist in internal medicine and infectiology, or a specialist in microbiology, virology and epidemiology of infectious diseases. In case of unavailability of the above-mentioned groups of specialists at the time of use, a specialist with appropriate experience in the treatment of infectious diseases with multidrug-resistant pathogens must be consulted.

Notes on pathogen detection

The reserve antibiotic may only be used as part of a targeted therapy. Before use, the causative pathogen and sensitivity to pathogen must always be proven by microbiological diagnostics of appropriate clinical material.

A calculated (empirical) use of dalbavancin without pathogen detection should only be carried out in exceptional cases. These include a known resistance problem in the treatment facility or in the case of a transfer from a facility with a known resistance problem, as well as in the case of a lack of therapeutic response to standard antibiotic therapy for a serious infection and urgent suspicion that the infection is caused by MRSA.

Samples for pathogen detection must be taken before the start of treatment. The calculated therapy must usually be adjusted after a maximum of 72 hours, if necessary, when an antibiogram is available.

Dalbavancin may not be used if the pathogen shows sensitivity to other antibiotics (without reserve status), unless other antibiotics cannot be used, for example because of contraindications or expected severe complications.

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Notes on implementation

The current guidelines of the AWMF and medical and, if applicable, also international scientific-medical societies for the appropriate use of antibiotics must be taken into account.

Furthermore, reference should be made to the listed requirements for a quality-assured application of dalbavancin in the locally available treatment guidelines and regulations of the measures for restrictive antibiotic use.

Outpatient implementation:

MRSA patients must be appropriately handled and the corresponding requirements in the practice must be met in accordance with the current recommendations of the RKI (Robert Koch Institute).

In order to ensure the overarching surveillance of resistance data on dalbavancin, the participation of the laboratory supporting the medical practice in ARS (antibiotic resistance surveillance) should be guaranteed.

Inpatient implementation:

The aforementioned specifications are to be implemented within the framework of regulations of the hospital's Drug Commission. Implementation should take place in particular within the framework of the in-house Antibiotic Stewardship Programme (ABS).

The treatment facility must have a local clearance policy for the use of dalbavancin in the respective treatment facility.

Consumption and resistance surveillance in accordance with Section 23 paragraph 4 Infection Protection Act (IPA) must be implemented. This is to be done through participation in the AVS (Antibiotic Consumption Surveillance) and ARS (Antibiotic Resistance Surveillance) or ARVIA (ARS and AVS – Integrated Analysis) systems.

The reporting of consumption and resistance data on dalbavancin to the above-mentioned systems should be ensured within six months of the entry into force of this resolution. Until participation in the mentioned systems, consumption and resistance situation must be ensured via the existing systems.

The principles of antibiotic therapy of the Commission on Anti-infectives, Resistance and Therapy (ART) at the Robert Koch Institute must be observed (last access: 06.06.2025):

https://www.rki.de/DE/Themen/Infektionskrankheiten/Antibiotikaresistenz/Kommission-ART/Weitere-Dokumente/Grundsaetze-der-Therapie.html?nn=16778978

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4. Treatment costs

Annual treatment costs:

Children from birth to < 3 months with acute bacterial skin and skin structure infections (ABSSSI)

Designation of the therapy	Annual treatment costs/ patient
Medicinal product to be assessed:	
Dalbavancin	€ 917.22

Costs after deduction of statutory rebates (LAUER-TAXE® as last revised: 15 September 2025)

Costs for additionally required SHI services: not applicable

Other SHI services:

Designation of the therapy	Type of service	Costs/ unit	Number/ cycle	Number/ Number/ patient/ year	Costs/ patient/ year
Dalbavancin	Surcharge for the preparation of an infusion solution containing antibiotics and virustatics	€ 39	1	1	€ 39

5. Designation of medicinal products with new active ingredients according to Section 35a, paragraph 3, sentence 4 SGB V that can be used in a combination therapy with the assessed medicinal product

In the context of the designation of medicinal products with new active ingredients pursuant to Section 35a, paragraph 3, sentence 4 SGB V, the following findings are made:

Children from birth to < 3 months with acute bacterial skin and skin structure infections (ABSSSI)

- No designation of medicinal products with new active ingredients that can be used in combination therapy in accordance with Section 35a, paragraph 3, sentence 4 SGB V, as the G-BA has decided to exempt the assessed medicinal product as a reserve antibiotic in accordance with Section 35a, paragraph 1c, sentence 1 SGB V.

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The designation of combinations exclusively serves the implementation of the combination discount according to Section 130e SGB V between health insurance funds and pharmaceutical companies. The findings made neither restrict the scope of treatment required to fulfil the medical treatment mandate, nor do they make statements about expediency or economic feasibility.

II. The resolution will enter into force on the day of its publication on the website of the G-BA on 20 November 2025.

The justification to this resolution will be published on the website of the G-BA at www.g-ba.de.

Berlin, 20 November 2025

Federal Joint Committee (G-BA)
in accordance with Section 91 SGB V
The Chair

Prof. Hecken

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